

Discharge Summary USE BLACK INK ONLY

Patient Details Surname Okafor Forename David M / F/ (Male)..... Date of Birth 09/05/1965 NHS/ Hosp No. 765 233 9081 Address 22 Elmfield Road, Woking, GU22 7TN Tel No. 01483 556 9012		Admission and GP Details Discharging Consultant Dr R. Fenwick Discharging Speciality/ Department Cardiology Method of Admission Emergency - ambulance via A&E Date of Admission 19/07/2026 Date of Discharge 23/07/2026 G.P. Details Dr H. Iqbal, Woking Health Centre	
Diagnosis at Discharge NSTEMI; single-vessel coronary disease (proximal RCA). PMH: hypertension, T2DM, hypercholesterolaemia, ex-smoker.		Operations and Procedures Coronary angiogram + PCI to proximal RCA with drug-eluting stent (20/07/2026), radial access, uncomplicated.	
Reason for Admission and Presenting Complaint(s) Acute onset central crushing chest pain radiating to left arm and jaw with sweating and nausea, at rest. Pre-hospital ECG showed ST depression inferolaterally.			
Clinical Narrative 61M with hypertension, T2DM, hypercholesterolaemia and prior smoking history (quit 2019) presented with typical cardiac chest pain. Troponin markedly elevated (peak 4500ng/L) with dynamic ECG changes, diagnosed with NSTEMI. Commenced on dual antiplatelets, fondaparinux, high-dose statin and beta-blocker per ACS pathway. Angiography demonstrated a severe proximal RCA stenosis, successfully treated with a single drug-eluting stent via radial approach. Recovery uncomplicated, radial band removed without haematoma. Echo showed preserved LV function (EF 55%). Referred to cardiac rehabilitation; glucose control optimised with diabetes team input.			
Relevant Investigations and Results Trop T 4500 (peak). Echo: EF 55%, no RWMA. Angiogram: severe proximal RCA stenosis, stented. HbA1c 68, LDL 3.9.			
Discharge Destination Home, lives with wife, fully independent, no care needs.			
Relevant legal Information (e.g. was an independent Mental Capacity Act Advocate required) No safeguarding or Mental Capacity Act concerns; full capacity.			
Information given to patient and/or authorised representative (including e.g. see GP in 2 weeks) Diagnosis, PCI outcome and secondary prevention discussed with patient and wife. Counselling on dual antiplatelet compliance, DVLA driving restriction (1 week), cardiac rehab, and lifestyle changes. Advised to call 999 for recurrent chest pain unrelieved by GTN.			
Physical Ability & Cognitive Function :		On Admission	At Discharge
Physical	Independent	Independent, mobilising well	
Cognitive	Normal	Normal	
Other	Chest pain	Asymptomatic, radial puncture site clean	
Advice, recommendations and future plans (including results awaited and outstanding investigations) G.P. Actions (Date) Check U&E and lipid profile in 4-6 weeks; review diabetes control. For GP: continue dual antiplatelets 12 months post-PCI - do not stop early without cardiology input. Up-titrate ACEi/beta-blocker; recheck lipids in 6-8 weeks (target LDL<1.4); HbA1c 68 - arrange diabetes review. Counselling given as above. Leaflet on heart attack, stenting and dual antiplatelet therapy provided. Strategies for potential problems GTN spray provided with instructions; wife aware of symptoms to watch for.			

